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TO: DAGGS, S REFERRAL & NOTES

FAX: 977074349725

SUBJECT:

Number of pages including this cover sheet: 9

Date and time of transmission: 5/8/2026 11:37:12 AM

FROM:

David Grant Medical Center (DGMC)
101 Bodin Circle
Travis AFB, CA 94535

FAX: 707-423-7535, DSN: 799-7535

Phone: 707-423-7641, DSN: 799-7641

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0014C-David Grant Medical Center
101 BODIN CIRCLE
TRAVIS AFB, CA 94535-

(707) 423-3000

Patient Name:	**Daggs, Steven E**	Admit:	4/15/2026 11:06 PDT
MRN:	2397424000001	Discharge:	4/15/2026 23:59 PDT
FIN:	202100144	Admitting:	
Sex/DOB/Age:	Male 9/9/1959 66 years	Location:	0014C-ALL-IMMUN
DOD ID (EDIPI):	1121253107	Veterans ID (ICN):	

Letters

Document Type:	Provider Letter
Service Date/Time:	4/29/2026 16:19 PDT
Result Status:	Auth (Verified)
Perform Information:	MINOR, ANNEKA M (4/29/2026 16:19 PDT)
Sign Information:	

DoD Military Treatment Facility
Defense Health Agency-TRICARE
7700 Arlington Blvd, Falls Church VA
22042

Re: Steven Daggs
Date of Visit: 04/15/2026

Dear Network Provider,

Below you will find information relevant to the referral order.

Referral Details
Medical Service: Physical Therapy
Referral Reason: 66yo M w/R shoulder pain likely rotator cuff tendinopathy, please evaluate and treat
Codified Reason: M25.511 - Pain in right shoulder
Order Comment:

--This Referral Is Electronically Signed by the Ordering Provider--
Referral Type: Evaluate and Treat (DoD)
Refer from Provider: CHERUBINI, MARY-KATHARINE, M.D.-Resident

This information has been disclosed to you from federal health records in accordance with an approved written request. Unless you are the subject of the record, federal laws may prohibit you from making a further disclosure of this record for a purpose other than why the information was provided to you. If this record contains substance abuse information, it is protected by 42 CFR Part 2 and/or 38 U.S.C. 7332 and the redisclosure restrictions of those federal laws on individuals who are not the subject of the record apply.

0014C-David Grant Medical Center

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Letters

Refer from Location: 0014C-David Grant Medical Center/0014C-ALL-IMMUN
Referral Written Date: 04/28/2026
Requested Start Date: 04/28/2026
Priority: Routine

Insurance
Health Plan: TRICARE PLUS
Payer: TRICARE
Financial Class: Tricare
Subscriber: Daggs, Steven E
Member Number: 00465688500
Group Number: 346
Begin Date: 07/17/2025

Other Insurances
Health Plan: 602 Direct Care and TRICARE Mail Order and Retail Pharm
Payer: TRICARE
Financial Class: Tricare
Subscriber: Daggs, Steven E
Member Number: 00465688500
Group Number: 602
Begin Date: 07/17/2025

Additional Referral Information
Patient ID: 1121253107
Patient DOB: 09-SEP-1959
Patient Gender: Male
Patient Phone: 7077184210
Sponsor DOD ID: 1121253107
Case Number: 626441871
Provider NPI: 1851091193
Defer to Network Reason: CP: MTF capacity, .
Instructions to Staff:
Treatment to Date:

Daggs, Steven E
2191 REVIVAL LN

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0014C-David Grant Medical Center

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Letters

RIO VISTA, CA 945712313

This referral order was electronically signed by the Refer from Provider listed above and is on file in the patient's electronic health record.

Please fax visit reports to the Military Treatment Facility Referral Management Center.

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0014C-David Grant Medical Center
101 BODIN CIRCLE
TRAVIS AFB, CA 94535-

(707) 423-3000

Patient Name:	**Daggs, Steven E**	Admit:	4/6/2026 15:16 PDT
MRN:	2397424000001	Discharge:	4/6/2026 23:59 PDT
FIN:	200739061	Admitting:	
Sex/DOB/Age:	Male 9/9/1959 66 years	Location:	0014C-FM-FMRC
DOD ID (EDIPI):	1121253107	Veterans ID (ICN):	

Office and Clinic Notes

Document Type:	Primary Care Note
Service Date/Time:	4/6/2026 15:16 PDT
Result Status:	Modified
Perform Information:	KNOBLOCH, ALEXANDER C, MD, Family Medicine, Sports Medicine (4/8/2026 15:07 PDT); CHERUBINI, MARY-KATHARINE, M.D.-Resident (4/6/2026 18:20 PDT)
Sign Information:	KNOBLOCH, ALEXANDER C, MD, Family Medicine, Sports Medicine (4/8/2026 15:07 PDT); CHERUBINI, MARY-KATHARINE, M.D.-Resident (4/6/2026 18:20 PDT)

Addendum by KNOBLOCH, ALEXANDER C, MD, Family Medicine, Sports Medicine on April 08, 2026 15:06:37 PDT
On the date of this encounter, I was immediately available to assist the resident in the care of the patient. I have reviewed and agree with the resident's findings and plan of care.

Alexander Knobloch, MD
Lt Col, USAF, MC
Family Medicine/Sports Medicine Faculty Physician
60 HCOS/SGGF
David Grant USAF Medical Center, Travis AFB, CA

Electronically signed on: 04/08/2026 15:07 PDT

ALEXANDER C KNOBLOCH
MD, Family Medicine, Sports Medicine

CHIEF COMPLAINT
R shoulder pain

ASSESSMENT/PLAN
1. Pain in right shoulder
Acute on chronic, new, mild and intermittent. Leading ddx: rotator cuff tendinopathy (given pain with shoulder flexion, and some special testing positive). Considered following ddx: AC arthropathy (but no AC joint tenderness and Scarf's negative), cervical radiculopathy (but Spurling tests negative), glenohumeral osteoarthritis (but no shoulder girdle atrophy noted, no hx shoulder trauma), biceps tendinopathy (but Speed's negative).
- Discussed differential with patient

PROBLEM LIST/PAST MEDICAL HISTORY
Ongoing
Anterior cruciate ligament rupture
Borderline glaucoma
Erectile dysfunction
Goiter
Gout
Hemorrhoids
Herpes infection
Hyperlipidemia
Hypertension
Non-allergic rhinitis

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Office and Clinic Notes

- Placed referral for physical therapy for 6-8 weeks
- Discussed pain rules in detail including avoiding pretreating pain before workout
- May continue ibuprofen 600 mg TID prn and acetaminophen 600 mg TID prn
- Right shoulder XR ordered
- Follow up in 6-8 weeks if pain unresolved

Ordered:
XR Shoulder Complete 2+ Views Right

Mary-Katharine Cherubini, Capt, USAF, MC
Family Medicine Resident, R3
60th Medical Group, HCOS/SGGF
David Grant USAF Medical Center
Travis Air Force Base, CA

HISTORY OF PRESENT ILLNESS

Provider HPI:
66yo M presents to clinic with R shoulder pain
- Previously worked for law enforcement and would get into altercations in which his shoulders were imapcted. Reports he was a paratrooper.
- Denies history of injection in R shoulder and doesn't want any injections.
- Reports pain radiates from posterior deltoid to neck. He will feel a low grade electrical voltage pain.
- Reports that this has been going on for years. Reports when he raised his right arm to take his blood pressure reproduced his pain. He doesn't have pain with driving. Reports that his symptoms are worst in the evening.
- He deals with this symptoms every day. Denies range of motion issues.
- Denies hx previous injury, surgery or injection to right upper extremity
- Numbness: Had 1 episode of numbness when he slept on his shoulder wrong but not persistently
- Reports the OTC 600 mg Motrin and 600 Tylenol helped a lot but heating pad relieves the pain pretty well. Reports the pain is worse with laying on his right arm or when he's in cold temperature.

Behavioral Health Screening
Initial Depression Screen Score: 0 Score (04/06/2026)
GAD2 Score: 0 Score (04/06/2026)

Travel Screening
TRAVEL HISTORY: Recent Travel History: No

Annual Questions
PATIENT PREFERENCES: Annual Questions Review Date: 04/06/2026 CM Preferred Spoken Language: English CM Preferred Written Language: English Learning Style Pref Patient: Demonstration, Printed materials Cultural Religious Beliefs Impact Care: No Patient Preference Comments: Pt was verfied with full name and DOB(707)7184210
HEALTH LITERACY AND LEARNING NEEDS: SILS Score: Never Barriers to Learning: None evident

Pain Detail

Pain in lower leg
PVD - peripheral vascular disease
Senile cataract of both eyes
Tobacco dependence caused by cigarettes
Type 2 diabetes mellitus, uncontrolled

Historical
Acute renal failure due to NSAID use
Dermatitis herpetiformis

PROCEDURE/SURGICAL HISTORY
- ACL repair Jul2011
- inguinal herniorrhaphy
- left submandibular salivary gland excision
- rhinoplasty
- turbinoplasty

IN-OFFICE MEDICATIONS AND/OR IMMUNIZATIONS
No qualifying data available.

MEDICATIONS
acetaminophen 500 mg oral tablet, 1000 mg= 2 tab(s), Oral, TID, PRN
allopurinol 300 mg oral tablet, 300 mg= 1 tab(s), Oral, Daily, 3 refills
amLODIPine 10 mg oral tablet, 10 mg= 1 tab(s), Oral, Daily, 3 refills
Aquaphor topical ointment, 1 appl(s), Topical, BID, PRN, 3 refills
Aspirin Enteric Coated 81 mg oral delayed release tablet, 81 mg= 1 tab(s), Oral, Daily, 3 refills
Atrovent 21 mcg/inh nasal spray, 2 spray(s), Nostril-Both, BID, 3 refills
cetirizine 10 mg oral tablet, 10 mg= 1 tab(s), Oral, Daily, PRN, 3 refills
cholecalciferol 50 mcg (2000 intl units) oral tablet, 2000 unit(s)= 1 tab(s), Oral, Daily, 3 refills
famciclovir 500 mg oral tablet, See Instructions, 2 refills
fexofenadine 180 mg oral tablet, 180 mg= 1 tab(s), Oral, Daily, 3 refills
Flonase 50 mcg/inh nasal spray, 50 mcg, Nostril-Both, BID, 3 refills
hydroCHLOROthiazide 25 mg oral tablet, 25 mg= 1 tab(s), Oral, Daily, 3 refills

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Office and Clinic Notes

DVPRS Pain Scale: 4 - Distracts me, can do usual activities

Intake HPI:
HPI
No qualifying data available.
66 y/o M presents to clinic for R shoulder pain. Pt has been experiencing R shoulder pain for the past couple of years that has worsened within the last few months. Pt experiences a dull/shooting pain that radiates towards down his R wrist describing it as a 4 out of 10. Pt states the pain is worsened when laying on his R shoulder or cold temperature. Pt states he has been taking OTC naproxen that has not been helping and using a heating pad at home that does relieve most of the pain. Pt denies any prev injuries/ trauma. Pt denies any pain pressing on his shoulder and mentioned he did experience some numbness in his R arm that also caused his R wrist to cramp. Pt states the pain could possibly be due to working law enforcement or being in the military. Pt has no further concerns.

REVIEW OF SYSTEMS
Fever: No
Chills: No
Recent Weight Loss: No
Recent Weight Gain: No
Headache: No
Earache: No
Nasal Discharge: No
Nasal Blockage: No
Throat Pain: No
Chest Pain: No
Cough: No
Difficulty Breathing: No
Nausea: No
Vomiting: No
Abdominal Pain: No
Hematochezia: No
Diarrhea: No
Constipation: No
Urinary Urgency: No
Urinary Frequency: No
Back Pain: No
Lightheadedness: No

Pain
DVPRS Pain Scale: 4 - Distracts me, can do usual activities (04/06/2026)

PHYSICAL EXAM

Vitals & Measurements
T: 36.7 °C (Oral) **HR:** 89(Peripheral) **RR:** 16 **BP:** 129/67 **SpO2:** 97%
HT: 184 cm(Measured) **WT:** 98.4 kg(Measured) **BMI:** 29.06 kg/m2(Measured)

General: Alert and oriented, well nourished, no acute distress.
Eye: EOMI grossly

Jardiance 25 mg oral tablet, 25 mg= 1 tab(s), Oral, every morning, 3 refills
metFORMIN 1000 mg oral tablet, 1000 mg= 1 tab(s), Oral, BID, 3 refills
Potassium Chloride (Eqv-K-Tab) 20 mEq oral tablet, extended release, 20 mEq= 1 tab(s), Oral, BID, 3 refills
rosuvastatin 20 mg oral tablet, 20 mg= 1 tab(s), Oral, Daily, 3 refills
telmisartan 20 mg oral tablet, 20 mg= 1 tab(s), Oral, Daily, 3 refills
tirzepatide (Mounjaro) Prefilled Pen 5 mg/0.5 mL subcutaneous solution, 5 mg, SubCutaneous, every week, 1 refills
urea 20% topical cream, 1 appl(s), Topical, Daily, PRN, 11 refills
Wellbutrin SR 150 mg/12 hours oral tablet, extended release, See Instructions, Oral

ALLERGIES
atorvastatin (Muscle weakness)
Monopril (swollen airway, swollen face)
fosinopril (Angioedema)
ibuprofen (Decreased renal function)

SOCIAL HISTORY
Alcohol
Current (Last 12 months) Use:. Monthly or less Frequency:. 1 Average drinks per episode in last year:., 11/30/2023
Past (No use in last 12 months) Use:., 10/13/2021
Tobacco
Yes-current everyday cigarette user Cigarette use:. 0.25 Average PACKS per day: (10 cigarettes = 0.5 packs). 45 Total years of smoking cigarettes:. Never-other tobacco user (not cigarettes) Other Tobacco use:., 11/30/2023
Never-cigarette user Cigarette use:. Never-other tobacco user (not cigarettes) Other Tobacco use:., 12/28/2022
Yes - current everyday cigarette user, 3-4 cigarettes/day Cigarette use:., 10/13/2021
Smoking tobacco use: 10 or more cigs(1/2 pack or more)/day in last 30 days. Type: Cigarettes., 04/02/2021

FAMILY HISTORY

0014C-David Grant Medical Center

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Office and Clinic Notes

HENT: Normocephalic
Lungs: Even and unlabored respirations
CV: Warm and well perfused throughout
Musculoskeletal: Moving all extremities
Skin: Exposed skin is warm and dry with no rashes or lesions noted.
Psychiatric: Cooperative, appropriate mood and affect.

Right Shoulder

Inspection
Asymmetry: N
Atrophy: N

Palpation
TTP clavicle: N
TTP AC joint: N
TTP anterior shoulder: N
TTP posterior shoulder: N

ROM
Flexion: Intact (0-180) and non-painful
Extension: Intact (0-50) and non-painful
Abduction: Intact (0-180) and non-painful

Special Tests
Supraspinatus: full can slightly positive, empty can engative, drop arm test normal
Infraspintus: hornblower normal
Teres Minor: NI
Subscapularis: belly-press normal, lift-off slightly positive
Glenohumeral: OBrien's negative
Biceps: Yorgason's negative, Speed's positive a little
AC Joint: Scarf's normal
Spurling tests: negative

Distal NVI: Y

Cancer: Negative: 32 = Mother (OHI) and 33 = Father (OHI).
Heart attack: 32 = Mother (OHI).
Stroke: 33 = Father (OHI).

RECOMMENDATIONS
Health Maintenance
Pending (in the next year)
OverDue
Adult - Fall Risk Screening due 01/29/21 and every 1 years
Adult - Colorectal Cancer Screening Recommendation Gap due 04/22/21 One-time only
Adult - Alcohol Use Screening due 03/11/24 and every 1 years
Adult - Tobacco Use Pharmacotherapy due 04/29/24 and every 1 years
Diabetes - Foot Exam/Foot Check due 11/28/24 and every 1 years
Due
Adult - DoD PTSD Screening due 04/06/26 Unknown Frequency
Adult - Domestic Violence/Relationship Safety Screening due 04/06/26 Unknown Frequency
Adult - Food Insecurity Screening due 04/06/26 Unknown Frequency
Adult - Hepatitis C Screening due 04/06/26 One-time only
Adult - RSV Vaccination - Risk Older Adults due 04/06/26 Unknown Frequency
Adult - Sexual Orientation due 04/06/26 Unknown Frequency
Adult - Tobacco and Nicotine Use Screening Follow-Up due 04/06/26 Unknown Frequency
Adult - VA Homelessness Screening due 04/06/26 Unknown Frequency
Diabetes - Eye Exam due 04/06/26 Unknown Frequency
Due In Future
Diabetes - HgbA1c Monitoring not due until 02/27/27 and every 366 days
CKD - Glomerular Filtration Rate not due until 03/26/27 and every 1 years
Diabetes - Serum Creatinine or eGFR not due until 03/26/27 and every 1 years

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Office and Clinic Notes

HTN - Serum Creatinine not due until 03/26/27 and every 1 years
CKD - Urine Albumin to Creatinine Ratio (uACR) not due until 03/27/27 and every 366 days
Diabetes - Urine Albumin to Creatinine Ratio (uACR) not due until 03/27/27 and every 366 days
Adult - Depression Screening not due until 04/02/27 and every 1 years
Adult - VA Suicide Risk Screening not due until 04/02/27 and every 1 years
Adult - Body Mass Index not due until 04/03/27 and every 366 days
Adult - Tobacco Use Screening not due until 04/03/27 and every 366 days
Satisfied (in the past 1 year)
Satisfied
Adult - Body Mass Index on 04/06/26. Satisfied by GOROSPE, HUMBER, EMT
Adult - Depression Screening on 04/06/26. Satisfied by GOROSPE, HUMBER, EMT
Adult - VA Suicide Risk Screening on 04/06/26. Satisfied by GOROSPE, HUMBER, EMT

CARE PATHWAYS THIS VISIT

No Results Found

Electronically signed on: 04/06/2026 18:20 PDT

MARY-KATHARINE CHERUBINI
M.D.-Resident

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